

# SYNTHETIC PATIENT REPORT - FOR DEMO USE ONLY

Patient: John Demo      MRN: MRN-0001      DOB: 01 May 1970      Sex: Male  
Report: Discharge Summary / Lab Panel      Date: 12 Aug 2026

## CLINICAL HISTORY

58-year-old male with a 6-year history of Type 2 Diabetes Mellitus, hypertension and dyslipidaemia, admitted with fatigue and polyuria.

## LABORATORY FINDINGS

- HbA1c: 9.1 % (target < 7.0 %) - poorly controlled glycaemia
- Fasting blood glucose: 182 mg/dL
- Blood pressure: 148/92 mmHg (stage 2 hypertension)
- LDL cholesterol: 162 mg/dL, Triglycerides: 210 mg/dL
- eGFR: 74 mL/min/1.73m<sup>2</sup> - mild renal impairment, monitor
- Urine microalbumin: 42 mg/g creatinine - early nephropathy marker

## IMAGING

MRI brain: no acute infarct. Mild chronic small-vessel ischaemic changes. CT abdomen: unremarkable, no organomegaly.

## ASSESSMENT

1. Type 2 Diabetes Mellitus, poorly controlled (HbA1c 9.1 %).
2. Stage 2 hypertension, newly quantified.
3. Mixed dyslipidaemia.
4. Early diabetic nephropathy (microalbuminuria).

## PLAN

- Start metformin 500 mg twice daily, titrate to 1000 mg twice daily.
- Start lisinopril 10 mg once daily (renoprotective + BP control).
- Start atorvastatin 20 mg nightly.
- Low-carbohydrate diet, 30 minutes brisk walking daily.
- Repeat HbA1c, renal panel and urine microalbumin in 6 weeks.
- Ophthalmology referral for diabetic retinopathy screening.